

LCME Standard 6 — Competencies, Curricular Objectives, and Curricular Design

What Changed and What It Means for Our DCI Response

Prepared for the Curricular Affairs Team · 2027-28 DCI Cycle

The Big Picture

Standard 6 is primarily a refinement update. The standard name, element count, and overall organization are unchanged. Most edits are clarifications, year reference updates, or minor rewordings. The two meaningful exceptions are Element 6.1, which has two new narrative questions and new supporting documentation requirements centered on EPO linkage, and Element 6.3, which has been removed and relocated to Standard 7.

Overall character of changes

The dominant theme across Standard 6 is tightening the evidence requirements for program objective documentation and linkage. LCME is asking schools not just to state that objectives exist and are linked, but to demonstrate prospective use of EPOs and to show the formal review process. The parallel curriculum footnote cleanup (6.4, 6.5, 6.8) removes campus-level table requirements, simplifying reporting for multi-campus schools.

Element Overview

#	Element	Level	Summary
6 (Standard)	Competencies, Curricular Objectives, and Curricular Design	MINOR	Academic year references updated; glossary footnote removed; schematic requirement clarified for parallel curricula.
6.1	Program and Learning Objectives	MINOR	Two new narrative questions (e, f) and new supporting documentation on EPO linkage. Most substantive change in Standard 6.
6.2	Required Clinical Experiences	No change	No changes detected.
6.3	Self-Directed and Life-Long Learning	REMOVED	Removed entirely; content relocated to Element 7.6 (Practice-Based Learning and Improvement).
6.4	Inpatient/Outpatient Experiences	MINOR	Footnote simplified - campus removed from parallel curriculum table instruction.
6.5	Elective Opportunities	MINOR	Same footnote simplification as 6.4.
6.6	Service-Learning/Community Service	MINOR	Glossary footnote removed; narrative (b) simplified - participation data no longer requested.
6.7	Academic Environments	MINOR	Table column renamed from Program Sponsor to CME Program Sponsor(s).
6.8	Education Program Duration	MINOR	Two footnotes updated: clerkship phase clarified; parallel curricula reference simplified.

** 6.1 is labeled MINOR in the automated analysis, but the addition of two new narrative questions and new supporting documentation makes it the most action-intensive element in Standard 6.*

Element-by-Element Summary of Changes

Standard-Level Changes

No structural or definitional changes at the standard level. Four administrative updates apply across the standard-level supporting documentation tables:

Previous (2026-27)	New (2027-28)
Academic year examples in Tables 6.0-1 and 6.0-2	Updated from 2025-26 to 2026-27 throughout.
Parallel curriculum data year	Updated from 2024-25 to 2025-26.
Curriculum schematic requirement	Updated from 2025-26 to 2026-27; footnote about LCME glossary definition of parallel curriculum removed.
Schematic of "any parallel curricula" changed to "each parallel curriculum"	Schools with more than one parallel curriculum must provide a schematic for each one, not a single combined schematic.

6.1 Program and Learning Objectives	MINOR CHANGE
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What Changed

Element 6.1 carries the most substantive changes in Standard 6. While the element definition is unchanged, three new requirements deepen the evidentiary expectations around program objective documentation and linkage. Two existing narrative questions were also updated.

1. Table terminology: outcome measures changed to types of assessment

Previous (2026-27)	New (2027-28)
Table instruction asks for outcome measure(s) used to assess students attainment of each EPO	Table instruction now asks for types of assessment used to assess students attainment of each EPO
Table column header: Assessment Measure(s) for Each Objective	Table column header: Types of Assessment for Each Educational Program Objective

The shift from outcome measures to types of assessment aligns with DCI-wide terminology. Ensure Table 6.1-1 column headers and instructions use the new language.

2. Updated narrative questions

Previous (2026-27)	New (2027-28)
(a) Provide the year in which the current medical education program objectives were last reviewed and approved.	(a) Provide the year in which the current medical education program objectives were last reviewed and by what group(s) they were approved.
(d) Summarize student agreement with the utility of the medical education program objectives.	(d) Summarize student agreement with the utility of the medical education program objectives to support their learning.

3. Two entirely new narrative questions

Q	New Question (did not exist in 2026-27 DCI)
e.	Describe and provide at least one example of how the medical education program objectives have been used prospectively in making a curriculum content change at the level of a course/clerkship or a curriculum phase.

	What this means: LCME wants to see that EPOs drive curriculum decisions - not just that EPOs exist. The example must show EPOs being consulted before a change was made, not as post-hoc justification.
f.	Describe whether course and clerkship objectives have been linked to the medical education program objectives, including when a curriculum change has been made. Summarize the roles and activities of course/clerkship faculty and the curriculum committee and its subcommittees in making and reviewing this linkage and note how often the linkage is reviewed. What this means: This question asks you to document the ongoing governance process for maintaining EPO-to-course-objective linkage - who does it, when, and how often. It also specifically asks whether linkage is updated when curriculum changes occur.

4. New supporting documentation required

Submit one example from a pre-clerkship course and one example from a clerkship that illustrates the linkage of all the learning objectives of that course/clerkship to the relevant medical education program objective(s).

The examples must show full linkage - all learning objectives, not a sample. One pre-clerkship course and one clerkship are both required.

What Your DCI Response Needs to Address

- Update Table 6.1-1 column header to Types of Assessment for Each Educational Program Objective and update table instructions to match.
- Narrative (a): add which group(s) approved the EPOs alongside the year of last review.
- Narrative (d): add reference to utility to support their learning.
- Narrative (e) - NEW: identify a concrete example where EPOs were consulted prospectively before a course/clerkship or phase-level change was made.
- Narrative (f) - NEW: describe the formal governance process for linking course/clerkship objectives to EPOs - who performs the linkage, when it is reviewed, and what happens when curriculum changes occur.
- Supporting documentation - NEW: prepare one pre-clerkship course and one clerkship document showing all learning objectives linked to EPOs.

6.3 Self-Directed and Life-Long Learning

REMOVED

✗ This element has been removed entirely from Standard 6

Element 6.3 has been relocated to Standard 7 as Element 7.6 (Practice-Based Learning and Improvement). Do not respond to 6.3 in the 2027-28 DCI. If your school previously had a strong 6.3 response, the content - particularly documentation of self-directed learning activities and the SDL assessment form - should be carried forward into your 7.6 response.

Content has moved, not disappeared

The substantive requirements for self-directed learning are now in Element 7.6 and are more detailed than the previous 6.3. See the Standard 7 brief for the full requirements, including the specific LCME definition of SDL that must be addressed and the new SDL assessment form submission requirement.

6.4 Inpatient/Outpatient Experiences

MINOR CHANGE

What Changed

Two small but deliberate changes that reduce the reporting burden for multi-campus schools and clarify governance language.

Previous (2026-27)	New (2027-28)
<i>Table footnote: Complete a separate table for each parallel curriculum and campus.</i>	If relevant, complete a separate table for each parallel curriculum. (Campus removed.)
<i>Narrative (a): mechanisms used by the curriculum committee, a curriculum subcommittee, or another authority</i>	Narrative (a): mechanisms used by the curriculum committee, a curriculum subcommittee, and/or another authority

First change: schools with a single parallel curriculum across multiple campuses no longer need separate tables per campus - one table per parallel curriculum is sufficient. Second change: and/or acknowledges that multiple oversight bodies may jointly determine inpatient/outpatient balance.

What Your DCI Response Needs to Address

- If your school has parallel curricula, one table per parallel curriculum is now sufficient - no separate tables per campus.
- In narrative (a), if both a curriculum subcommittee and another authority are involved in determining inpatient/outpatient balance, name both.

6.5 Elective Opportunities

MINOR CHANGE

What Changed

One change, identical to the footnote update in 6.4:

Previous (2026-27)	New (2027-28)
<i>Complete a separate table for each parallel curriculum and campus.</i>	If relevant, complete a separate table for each parallel curriculum.

Same implication as 6.4: separate tables per campus are no longer required. One table per parallel curriculum is sufficient.

What Your DCI Response Needs to Address

- If applicable, reduce to one table per parallel curriculum rather than per campus.

6.6 Service-Learning/Community Service

MINOR CHANGE

What Changed

Two changes, both reducing prior requirements:

Previous (2026-27)	New (2027-28)
<i>Footnote: See the LCME Glossary for the definitions of service-learning and community service.</i>	Footnote removed entirely.
<i>Narrative (b) previously ended with: Provide school data, as available, on the level of students participation in service-learning and/or community service activities.</i>	Second sentence removed. Narrative (b) now asks only: How are students informed about the availability of service-learning and/or community service activities?

Student participation data is no longer requested in narrative (b). You still need to describe how students are informed about opportunities, but quantitative participation reporting has been dropped.

What Your DCI Response Needs to Address

- Narrative (b): describe how students are informed about opportunities only - do not include or collect participation data for this response.
- No glossary footnote to address.

6.7 Academic Environments

MINOR CHANGE

What Changed

One table column header update:

Previous (2026-27)	New (2027-28)
Column header: Program Sponsor	Column header: CME Program Sponsor(s)

The update clarifies that the column refers specifically to CME (Continuing Medical Education) program sponsors, and acknowledges there may be more than one. If your institution has multiple CME sponsors, list all of them.

What Your DCI Response Needs to Address

- Update the column header in your Table 6.7 response to CME Program Sponsor(s).
- If your institution has more than one CME program sponsor, list all of them.

6.8 Education Program Duration

MINOR CHANGE

What Changed

Two footnote updates clarifying terminology and simplifying parallel curriculum reporting:

Previous (2026-27)	New (2027-28)
Footnote 1: The clerkship phase includes the time for required clinical and other related activities.	Footnote 1: The clerkship phase includes the time for required clinical clerkships and other related activities.
Footnote 2: Note any differences for parallel curricula and/or campuses.	If relevant, note any differences for parallel curricula. (Campuses removed.)

The first change adds specificity: clinical clerkships is now explicit rather than the broader clinical activities. The second continues the standard-wide removal of campus-level footnote requirements.

What Your DCI Response Needs to Address

- No narrative changes required - these are footnote clarifications only.
- If noting duration differences for parallel curricula, do so at the curriculum level, not by individual campus.

Summary Action Checklist for the Curricular Affairs Team

Use this checklist when preparing your Standard 6 responses for the 2027-28 DCI.

Academic Year and Administrative Updates (Standard Level)

- ☐ Update academic year references in Tables 6.0-1 and 6.0-2 to 2026-27.
- ☐ Update the parallel curriculum data year reference to 2025-26.
- ☐ Update the curriculum schematic requirement to reflect 2026-27.
- ☐ If your school has multiple parallel curricula, prepare a separate schematic for each one.

EPO Documentation and Linkage (6.1)

- ☐ Update Table 6.1-1: rename column header to Types of Assessment for Each Educational Program Objective and update table instructions.
- ☐ Narrative (a): add which group(s) approved the EPOs, not just the year.
- ☐ Narrative (d): add to support their learning to the EPO utility discussion.
- ☐ Narrative (e) - NEW: document a concrete example of EPOs used prospectively before a curriculum change.
- ☐ Narrative (f) - NEW: describe the governance process for linking course/clerkship objectives to EPOs - who, when, and how often.
- ☐ Supporting documentation - NEW: prepare one pre-clerkship course and one clerkship document showing all learning objectives linked to EPOs.

Removed Element (6.3)

- ☐ Do not include a 6.3 response - this element no longer exists.
- ☐ Carry any strong 6.3 content forward into Element 7.6 (Practice-Based Learning and Improvement).
- ☐ See the Standard 7 brief for the full requirements of the new 7.6.

Parallel Curriculum Footnote Simplification (6.4, 6.5, 6.8)

- ☐ 6.4 and 6.5: reduce to one table per parallel curriculum - no longer need separate tables per campus.
- ☐ 6.8: if noting duration differences for parallel curricula, do so at the curriculum level, not by individual campus.
- ☐ If your school has no parallel curricula, the if relevant qualifier means these footnotes do not apply.

Minor Wording and Table Updates (6.6, 6.7)

- ☐ 6.6 narrative (b): describe how students are informed about opportunities only - remove any participation data.
- ☐ 6.7 table: update column header from Program Sponsor to CME Program Sponsor(s); list all CME sponsors if more than one.

LCME Standard 7 — Curricular Content

What Changed and What It Means for Our DCI Response

Prepared for the Curricular Affairs Team · 2027–28 DCI Cycle

The Big Picture: Why Standard 7 Looks Different

Standard 7 has undergone the most significant structural overhaul in the 2027–28 DCI. This is not a minor update — it is a deliberate realignment of LCME curricular content standards to match the ACGME/ABMS core competency framework used across graduate medical education.

⚠️ What drove this change?

The 2026–27 DCI organized Standard 7 around content domains (biomedical sciences, organ systems, ethics, societal problems). The 2027–28 DCI reorganizes it around physician competencies — the same framework residents are evaluated on. This signals that LCME now expects medical schools to demonstrate that their curriculum is explicitly building toward ACGME competency readiness, not just covering content areas.

The standard-level definition itself changed. The new language adds three key phrases:

New Phrase	What It Signals
"through its curriculum governance process"	Faculty must demonstrate that Standard 7 content decisions flow through formal governance — not just that the content exists.
"content and experiences" (not just "content")	Clinical experiences are now explicitly required alongside didactic content for each competency domain.
"graduate medical education program" (not "residency program")	Scope expanded to include fellowship and other GME pathways.

Element Mapping: Old → New

Nine elements in the 2026–27 DCI have been consolidated and reorganized into seven elements aligned with ACGME core competencies. Note that 7.8 and 7.9 no longer exist as standalone elements.

#	2026–27 DCI Element	2027–28 DCI Element
1	7.1 Biomedical, Behavioral, Social Sciences	7.1 Foundational Medical Knowledge
2	7.2 Organ Systems/Life Cycle/Prevention/Symptoms/Signs/Differential Diagnosis, Treatment Planning	7.2 Patient Care
3	7.3 Scientific Method/Clinical/Translational Research	7.3 Health Promotion and Health Maintenance
4	7.4 Critical Judgment/Problem-Solving Skills	7.4 Communication and Interprofessional Collaborative Skills

5	7.5 Societal Problems	7.5 Professionalism
6	7.6 Structural Competence, Cultural Competence and Health Inequities	7.6 Practice-Based Learning and Improvement
7	7.7 Medical Ethics	7.7 Systems-Based Practice
8	<i>7.8 Communication Skills</i>	→ Consolidated into 7.4
9	<i>7.9 Interprofessional Collaborative Skills</i>	→ Consolidated into 7.4

✕ Removed Elements

7.8 Communication Skills and 7.9 Interprofessional Collaborative Skills have been eliminated as standalone elements. Their content has been absorbed into Element 7.4 (Communication and Interprofessional Collaborative Skills).

Element-by-Element Summary of Changes

7.1 Foundational Medical Knowledge

Previously: Biomedical, Behavioral, Social Sciences

What Changed

The content scope of this element is largely unchanged — the biomedical, behavioral, and socioeconomic sciences still form the core. The major shift is in how evidence of that teaching is reported, and what additional topics are now required.

The biggest table change: LCME has moved away from AAMC Graduation Questionnaire (GQ) data as the primary evidence and now requires schools to report internal student assessment (ISA) data by organ system and by discipline — broken down by curriculum year. Two new tables (7.1-1 and 7.1-2) require organ system and discipline-specific student satisfaction data from your own internal survey.

Research education is now explicitly captured here: A new Table 7.1-4 requires three learning objectives for each of four research topics (basic science research, clinical research, translational research, and application to patient care), along with types of assessment for each. This content was previously in Element 7.3.

New supporting documentation required: NBME subject area performance graphs comparing national and school first-time takers for USMLE Step 1 for the past three years must now be submitted.

What Your DCI Response Needs to Address

- Run and report ISA data by organ system (12 systems) and by discipline — broken down by curriculum year.
- Confirm your ISA survey instrument includes the new organ system and discipline items; update it if not.
- Map your research curriculum (basic science, clinical, translational) to 3 learning objectives each and identify assessment types.
- Obtain and submit the NBME Step 1 subject area performance comparison graphs for the past 3 years.
- Narrative question (a) now asks you to note concerns from the ISA data and describe how you are addressing them — not just identify content gaps.

7.2 Patient Care

Previously: Organ Systems/Life Cycle/Prevention/Symptoms/Signs/Differential Diagnosis, Treatment Planning

What Changed

This element has been substantially rewritten in both title and scope. The 2026–27 focus on listing organ systems, life cycle phases, and care types has been replaced with a competency framing around diagnosis, treatment, and the application of clinical tools.

Lifespan content is now a table (not a narrative): Table 7.2-3 asks you to mark curriculum phases where continuity of care, preventive care, acute care, chronic care, rehabilitative care, and end-of-life care are taught and assessed.

Emerging technology and AI are now explicitly required: New Table 7.2-4 asks for learning objectives on the appropriate use of AI in disease diagnosis and management, with space to add other emerging technologies.

Evidence-based medicine is now its own table (7.2-5): Three learning objectives are required for "Principles of EBM" and "Application of EBM to clinical decision-making," with associated courses/clerkships and assessment types.

Supporting documentation has changed: NBME Step 2 CK performance graphs and the school's most recent Resident Readiness Survey (or equivalent) are now required attachments — replacing the old GQ-based tables.

What Your DCI Response Needs to Address

- Map your diagnosis/treatment curriculum in Tables 7.2-1 and 7.2-2 using ISA data.
- Complete Table 7.2-3 showing where lifespan care types (preventive, acute, chronic, rehabilitative, end-of-life) appear in the curriculum.
- Develop 1–2 learning objectives on AI use in diagnosis and management (Table 7.2-4); expand for other emerging technologies if applicable.
- Document EBM objectives with courses and assessment types for Table 7.2-5.
- Obtain NBME Step 2 CK graphs and your Resident Readiness Survey results for the supporting documentation section.
- Narrative question (a) now has four sub-areas to address: diagnosis/management, lifespan care, AI/emerging tech, and EBM.

7.3 Health Promotion and Health Maintenance

Previously: Scientific Method/Clinical/Translational Research

What Changed

This is a near-complete replacement. The 2026–27 element 7.3 focused entirely on research methods and research ethics. That content has been moved to Element 7.1 (Table 7.1-4). Element 7.3 now covers health promotion, disease prevention, nutrition, and the health impact of societal problems — content that was previously scattered across other elements.

Four new ISA-based tables: Tables 7.3-2 through 7.3-5 each ask for ISA data by curriculum year on: (1) health promotion/disease prevention, (2) health maintenance, (3) nutrition across the life cycle, and (4) challenges to wellness from societal problems.

Societal problems now appear here, not in 7.5: Narrative question (a) asks you to name three common societal problems the school has identified, and for each: describe the relationship to personal wellness, the phases where it is taught, and the assessments used.

What Your DCI Response Needs to Address

- Complete Table 7.3-1 mapping health promotion, health maintenance, nutrition, and wellness challenges across curriculum phases.
- Confirm your ISA includes items on health promotion, health maintenance, nutrition, and societal wellness challenges; update if needed.
- Select 3 societal problems the school explicitly addresses and document their phase placement and assessment methods for narrative (a).
- Note: If your school previously reported research methods content under 7.3, that content now belongs in 7.1 Table 7.1-4.
- Nutrition content is now explicitly required — confirm it appears in your curriculum map.

7.4 Communication and Interprofessional Collaborative Skills

Previously: Critical Judgment/Problem-Solving Skills

What Changed

This element absorbs the content of the now-eliminated Elements 7.8 (Communication Skills) and 7.9 (Interprofessional Collaborative Skills) and combines them under a single competency domain. The previous element 7.4 on critical judgment and problem-solving has been removed; that content now lives in the standard-level framing and in 7.2 (Patient Care).

Communication now requires mapped learning objectives (Table 7.4-1): Three learning objectives are required for each of three communication topics: communicating with patients and families, communicating with physicians, and communicating with non-physician health professionals.

IPE has a new phase-mapping table (7.4-2): You must show where students are brought together with students or practitioners from other health professions to learn team roles, effective team functioning, and collaborative care delivery — by curriculum phase.

Narrative question (a) is now about IPE examples, not problem-solving: Provide one curricular example each for: team roles, effective team functioning, and collaborative care delivery. Each example must include the teaching format, curriculum phase, disciplines of other health professional students/practitioners, and assessment methods.

What Your DCI Response Needs to Address

- Map 3 learning objectives for each of the 3 communication topics in Table 7.4-1 and identify assessment types.
- Complete Table 7.4-2 identifying where IPE experiences occur by phase for team roles, team functioning, and collaborative care.
- Prepare 3 IPE curricular examples (one per category) with full details: format, phase, disciplines, and assessment.
- Content previously reported under 7.8 and 7.9 should now be incorporated here — do not duplicate across elements.
- If ISA data show concerns about communication or IPE readiness, address these in narrative question (b).

7.5 Professionalism

Previously: Societal Problems

What Changed

This element is a consolidation of multiple previously separate content areas. The old "Societal Problems" focus has been moved to 7.3. Element 7.5 now brings together professionalism, human values, medical ethics (previously 7.7), and cultural awareness — creating a unified "Professionalism" competency element.

Professional behaviors are now listed and mapped (Table 7.5-1): You must list the professional behaviors students are expected to learn and demonstrate, identify the phase(s) where each is addressed, and specify the assessment types used.

Medical ethics now requires 3 learning objectives per topic (Table 7.5-2): Three topics are required: biomedical ethics, ethical decision-making, and ethical behavior in patient care — each with three mapped learning objectives and assessment types.

Cultural awareness is now an explicit table (7.5-3): Three learning objectives are required for each of two topics: cultural awareness and respect, and incorporation of cultural awareness into interactions with patients and families. This content was previously captured in Element 7.6.

Two new ISA tables: Tables 7.5-4 and 7.5-5 require ISA data by curriculum year on: (1) student preparation to understand the ethical and professional values expected of a physician, and (2) student preparation to care for patients from different backgrounds.

What Your DCI Response Needs to Address

- List all expected professional behaviors with phase and assessment type mapping for Table 7.5-1.
- Map 3 learning objectives each for biomedical ethics, ethical decision-making, and ethical behavior in patient care (Table 7.5-2).
- Map 3 learning objectives each for cultural awareness/respect and its application to patient interactions (Table 7.5-3).
- Confirm your ISA covers the two new professionalism and cultural awareness items; update if needed.

- Narrative question (b) asks specifically how professional/ethical breaches are identified, remediated, or referred — include process details.
- Content previously reported in 7.7 (medical ethics) and 7.6 (cultural competence) now belongs here.

7.6 Practice-Based Learning and Improvement

Previously: Structural Competence, Cultural Competence and Health Inequities

What Changed

The previous Element 7.6 on structural competence, cultural competence, and health inequities has been reorganized. Cultural competence content moves to 7.5 (Professionalism). Health disparities content moves to 7.7 (Systems-Based Practice). Element 7.6 now covers Practice-Based Learning and Improvement — absorbing the content previously in the eliminated Element 6.3 (Self-Directed and Life-Long Learning).

LCME's specific definition of self-directed learning applies here: The element explicitly references the LCME definition of self-directed learning, which requires: (1) independent self-assessment of learning needs, (2) independent identification, analysis, and synthesis of relevant information, (3) independent self-assessment of the credibility of information sources, and (4) facilitator assessment and feedback on each of these skills — all occurring in a unified sequence over a relatively short time period.

Table 7.6-1 asks specifically about SDL practices: ISA data is required on whether "the curriculum provides practice in the skills of self-directed learning as defined by the LCME." A second table (7.6-2) asks whether "the curriculum prepared me to use the skills of self-directed learning in the clerkships."

New supporting documentation: A sample form used to assess the development of students' self-directed learning skills must be submitted.

What Your DCI Response Needs to Address

- Identify a specific learning activity that meets all four LCME SDL components in a unified sequence — this is the core of narrative question (a).
- The four SDL components (self-assessment of needs, information synthesis, source credibility, facilitator feedback) must each be explicitly named and described.
- Describe how students are helped to recognize strengths, deficiencies, and limitations in their professional development (narrative b).
- Submit a sample SDL assessment form as supporting documentation.
- Content previously in 7.6 (structural/cultural competence, health disparities) has moved to 7.5 and 7.7 — do not include it here.
- If ISA data show concerns about SDL readiness, address these in your narrative.

7.7 Systems-Based Practice

Previously: Medical Ethics

What Changed

The previous Element 7.7 (Medical Ethics) has been moved into Element 7.5 (Professionalism). Element 7.7 now covers Systems-Based Practice — addressing healthcare delivery systems, health outcome disparities, cost-effective care, and patient safety. This is new content that was not previously captured as a standalone element.

Three required topics, each with 3 learning objectives and instructional formats (Table 7.7-1): (1) Structure and functioning of healthcare delivery systems, (2) Determinants of disparate health outcomes for individuals, and (3) Determinants of disparate health outcomes for communities/populations — with resources needed to optimize outcomes for each.

Instructional formats are now required alongside assessment types: Unlike other elements that ask only for assessment types, Table 7.7-1 requires both instructional formats and types of assessment for each learning objective.

Narrative question (a) is now a gap/concern analysis: The question asks you to note any deficiencies or concerns about content in the three topic areas and summarize steps taken to address them. The previous narrative (a) asked where students are introduced to ethical principles, and narrative (b) on identifying ethical breaches has moved to 7.5.



What Your DCI Response Needs to Address

- Map 3 learning objectives for each of the 3 systems-based practice topics in Table 7.7-1, with both instructional formats and assessment types.
- Ensure content covers health disparities at both the individual and community/population level.
- Include content on resources needed to optimize health outcomes — this is explicitly part of the element definition.
- Narrative question (a) asks you to identify and address any gaps; review your curriculum map for coverage of all three topic areas.
- Ethics content previously reported here now belongs in 7.5 — do not duplicate.

Summary Action Checklist for the Curricular Affairs Team

Use this checklist when preparing your Standard 7 responses for the 2027–28 DCI.

Data & ISA Updates

- ☐ Update your ISA instrument to include organ system (12 systems) and discipline-specific items for 7.1.
- ☐ Confirm ISA includes items on health promotion, health maintenance, nutrition, and wellness for 7.3.
- ☐ Add ISA items on SDL readiness (7.6) and IPE collaborative skills (7.4) if not present.
- ☐ Add ISA items on professionalism and caring for patients from diverse backgrounds (7.5).
- ☐ Obtain NBME Step 1 subject area graphs (7.1) and Step 2 CK graphs (7.2) for 3 years.
- ☐ Obtain or prepare a Resident Readiness Survey report or equivalent (7.2).

Curriculum Mapping

- ☐ Map research education (basic, clinical, translational, application) to 7.1 Table 7.1-4.
- ☐ Map lifespan care types (preventive, acute, chronic, rehabilitative, end-of-life) to 7.2 Table 7.2-3.
- ☐ Confirm AI and EBM content exists and map to 7.2 Tables 7.2-4 and 7.2-5.
- ☐ Map health promotion, disease prevention, nutrition, and 3 societal problems to 7.3.
- ☐ Map communication learning objectives (patients/families, physicians, non-physician team) to 7.4.
- ☐ Map IPE experiences by phase to 7.4 Table 7.4-2.
- ☐ Map all professional behaviors with phases and assessment types to 7.5 Table 7.5-1.
- ☐ Map medical ethics and cultural awareness learning objectives to 7.5 Tables 7.5-2 and 7.5-3.
- ☐ Identify the SDL activity that meets all four LCME components for 7.6.
- ☐ Map systems-based practice objectives (3 topics) with instructional formats and assessments for 7.7.

Content Consolidations to Verify

- ☐ Research methods content (previously 7.3) is now reported in 7.1 Table 7.1-4.
- ☐ Communication skills (previously 7.8) are now in 7.4 — do not duplicate.
- ☐ IPE content (previously 7.9) is now in 7.4 — do not duplicate.
- ☐ Medical ethics content (previously 7.7) is now in 7.5.
- ☐ Cultural competence content (previously 7.6) is now in 7.5.
- ☐ Health disparities and health equity content (previously 7.6) is now in 7.7.
- ☐ Societal problems content (previously 7.5) is now in 7.3.

- Self-directed learning content (previously 6.3) is now in 7.6.

Supporting Documentation

- 7.1: NBME Step 1 subject area performance graphs (3 years).
- 7.2: NBME Step 2 CK performance graphs (3 years) + Resident Readiness Survey.
- 7.6: Sample SDL assessment form.

LCME Standard 8 — Curricular Management, Evaluation, and Enhancement

What Changed and What It Means for Our DCI Response

Prepared for the Curricular Affairs Team · 2027–28 DCI Cycle

The Big Picture

Standard 8 did not undergo the structural overhaul that Standard 7 did. The standard name, element count, and overall organization are unchanged. However, several targeted changes carry meaningful compliance implications — particularly for how the curriculum committee is constituted, how outcome thresholds are defined, and how comparability across sites is documented.

Overall character of changes

Most updates in Standard 8 are clarifications and tightenings rather than wholesale rewrites. The major exceptions are: (1) Element 8.1, where new faculty composition requirements for the curriculum committee have been made explicit; (2) Element 8.2, which has been removed entirely; (3) Element 8.4, where the National Norms of Accomplishment thresholds have been updated and NBME graph requirements moved; and (4) Element 8.7, where the comparability table has been eliminated and narrative questions significantly reorganized.

Element Overview

#	Element	Change Level	Key Change
8.1	Curricular Management	MINOR	Curriculum committee definition strengthened — new faculty composition and voting requirements made explicit.
8.2	Use of Medical Educational Program Objectives	REMOVED	Removed entirely. Content absorbed into other elements.
8.3	Curricular Design, Review, Revision/Content Monitoring	MINOR	Small wording clarifications; one new phrase added to narrative question (a).
8.4	Evaluation of Educational Program Outcomes	MAJOR	National Norms thresholds updated to rolling averages; NBME graphs moved to Standard 7; table column renamed.
8.5	Medical Student Feedback	MINOR	Institutional language updated; minor reframing of two narrative questions.
8.6	Monitoring of Completion of Required Clinical Experiences	MINOR	Minor clarifications to narrative questions (a) and (e).
8.7	Comparability of Education/Assessment	MAJOR	Supporting data table removed; narrative questions restructured from 3 to 5 questions.
8.8	Monitoring Student Time	MINOR	Element definition updated; two tables renamed; narrative question (g) reworded.

Element-by-Element Summary of Changes

8.1 Curricular Management

MINOR CHANGE

What Changed

The element definition has been tightened in three specific ways that carry structural compliance implications for how your curriculum committee is constituted and how it operates.

New Language	What It Requires
"standing faculty committee"	The committee must be a permanent, standing body — not ad hoc or periodically constituted.
"authority and responsibility" (previously just "responsibility")	The committee must have formal authority over the curriculum, not merely an advisory or coordinating role. Bylaws should reflect this.
"majority of voting members overall and at each meeting must be faculty with medical school appointments"	Applies both to total committee composition and to quorum at each individual meeting. Non-faculty members (students, administrators) cannot constitute the majority.
"at least one member from the category defined as general faculty"	The bylaws/policy must define a "general faculty" category, and this category must have at least one voting seat.
Narrative (b): "general faculty from basic science or clinical departments" added as a category example	Reflects the new distinction between general faculty and faculty in leadership roles (course/clerkship directors).
Supporting docs: hard copies now available "at the request of the survey team secretary" (not automatically)	Minor procedural clarification — reduces blanket printing burden but materials must still be ready.

What Your DCI Response Needs to Address

- Confirm your curriculum committee bylaws/policy explicitly grants the committee authority (not just responsibility) over curriculum design, management, and evaluation.
- Verify that faculty with medical school appointments constitute a majority of voting members both overall and at every quorum meeting.
- Confirm the bylaws define a "general faculty" category (typically non-leadership-role faculty) and that at least one such member holds a voting seat.
- In narrative question (b), list membership categories using the new terminology — distinguish "general faculty" from faculty holding leadership roles (course/clerkship directors).
- Hard copies of committee minutes: prepare them but submit only if specifically requested by the survey team secretary.

8.2 Use of Medical Educational Program Objectives

REMOVED

This element has been removed entirely from the 2027–28 DCI

Element 8.2 previously required documentation that the medical education program objectives (EPOs) were used by faculty in the design and review of courses and clerkships. This requirement has been absorbed into other elements — particularly 8.3 (curriculum content monitoring and linkage of course objectives to EPOs) and 8.4 (evaluation of whether students are achieving the EPOs). You no longer need

to respond to 8.2 as a standalone element, but the underlying expectation that EPOs drive curriculum design remains embedded in 8.3 and 8.4.

⚠ Watch for residual expectations in 8.3 and 8.4

Narrative question (d) in 8.3 still asks for two examples of how curriculum content monitoring and the review of course/clerkship objective linkages to EPOs identified gaps and redundancies. The EPO linkage expectation has not disappeared — it has just been folded into the content monitoring element.

8.3 Curricular Design, Review, Revision/Content Monitoring

MINOR CHANGE

What Changed

This element received focused wording clarifications that add specificity to what the narrative responses must address. No tables or structural changes.

What Changed	Implication for Your Response
<i>Narrative (a) now includes "including preparing students for the next phase of their training"</i>	Phase reviews must explicitly address whether each phase is preparing students for what comes next — not just whether the phase is meeting its own stated objectives.
<i>Narrative (a), item 1: "specific data and information sources"</i>	"Specific" is now in the language. Vague references to "data" will be insufficient; name the instruments and sources used.
<i>Narrative (a), item 3: now numbered "3."</i>	Minor formatting correction; no content change.
<i>Narrative (d): "linkage of...objectives and EPOs" changed to "linkage of...objectives to EPOs"</i>	Minor grammatical correction; no content change.

▶ What Your DCI Response Needs to Address

- In narrative (a), explicitly address how each phase review evaluates student preparation for the next phase (pre-clerkship → clerkship → post-clerkship).
- Name specific data sources — instruments, surveys, exam results, committee reviews — rather than describing processes in general terms.
- In narrative (d), confirm your two examples demonstrate EPO-to-course-objective linkage reviews, not just content gap identification.

8.4 Evaluation of Educational Program Outcomes

MAJOR CHANGE

What Changed

Several targeted but important changes affect how outcome threshold data are presented and what supporting documentation is required.

1. National Norms of Accomplishment thresholds updated

The fixed percentage benchmarks in the previous DCI have been replaced with dynamic thresholds that reference rolling national averages. This removes specific numbers from the standard (which could become outdated) and ties the thresholds to current national performance.

	2026–27 Language (Fixed Benchmarks)	2027–28 Language (Rolling Averages)
Step 1	<i>USMLE pass rate in Step 1 below 83%, which is 10% below the average pass rate over the most recent two years (93%)</i>	USMLE Step 1 first-time pass rate that is 10% below the first-time pass rate averaged over the most recent two years for which national data are available
Step 2 CK	<i>USMLE pass rate in Step 2 CK below 88%, which is 10% below the average pass rate over the most recent two years (98%)</i>	USMLE Step 2 CK pass rate that is 10% below the first-time pass rate averaged over the most recent two years for which national data are available
Attrition	<i>Total percent attrition during each of the last two academic years of 5% or greater per year (average total percent attrition during the most recent academic years is 1% per year)</i>	Total percent attrition during each of the last two academic years of 5% or greater per year
Match	<i>Initial residency Match rate of 83%, which is 10 percentage points below the average Match rate over the most recent two years (93%)</i>	Initial residency Match rate that is 10 percentage points below the Match rate averaged over the most recent two years

2. NBME graphs no longer required as supporting documentation here

The previous 8.4 required submission of NBME subject area performance graphs for USMLE Step 1 and Step 2 CK as supporting documentation. These have been removed from 8.4 and moved to Standards 7.1 and 7.2 respectively. Do not submit NBME graphs under 8.4 in the new DCI.

3. Minor language updates

"Outcome Indicator" column in Table 8.4-1 renamed to "Program Outcome Indicator." Narrative (a) now says "all the EPOs" (added "all") — emphasizing completeness. Narrative (c) now lowercases "national norms of accomplishment."

What Your DCI Response Needs to Address

- Update your outcome threshold tracking: the benchmarks are now relative to rolling national averages, not fixed percentages. Know your current national averages for Step 1, Step 2 CK, and Match rate.
- Do not include NBME Step 1 or Step 2 CK performance graphs in the 8.4 supporting documentation section — they now belong in 7.1 and 7.2.
- In narrative (a), confirm your EPO evaluation process covers all EPOs, not just a sample.
- In narrative (c), ensure your two outcome examples address current national norm thresholds rather than citing the old fixed percentages.
- Update Table 8.4-1: the first column header is now "Program Outcome Indicator."

8.5 Medical Student Feedback

MINOR CHANGE

What Changed

Three small but deliberate changes. Two shift the institutional subject from "medical school" to "medical education program," consistent with language updates across the DCI. One reframes a narrative question.

→ *Element definition: "medical school" → "medical education program"*

Reflects the DCI-wide language shift acknowledging that the education program may span multiple institutional entities.

→ *Narrative (c): "actions taken or not taken based on their input regarding courses and clerkships" → "actions taken or not taken on courses and clerkships based on their input"*

Subtle reordering — the focus is now on what was done to courses/clerkships, not just on communicating back to students. Emphasize tangible curricular actions.

→ *Narrative (d): "Discuss data from the ISA" → "Summarize the data from the ISA"*

"Summarize" implies a more structured presentation than "discuss." Include specific percentages and trends, not just qualitative commentary.

What Your DCI Response Needs to Address

- In narrative (c), describe specific changes made to courses or clerkships in response to student feedback, not just the communication process.
- In narrative (d), present ISA data as a structured summary with specific numbers and trends, not a general discussion.

8.6 Monitoring of Completion of Required Clinical Experiences

MINOR CHANGE

What Changed

Two minor clarifications — one removes a phrase from a narrative question, one adds benchmark language to a threshold description.

→ *Narrative (a): phrase "Note whether there is a centralized tool for logging or there are logging processes/tools at the departmental level" removed*

The logging description is simplified. You no longer need to explicitly distinguish between centralized vs. departmental logging tools — just describe the process used.

→ *Narrative (e): "more than 20% of cases" reframed as "the school has set more than 20% of cases in aggregate or at a particular site as its benchmark"*

This shifts from an implied universal threshold (>20%) to a school-defined benchmark. If your school has set a different internal threshold for alternate methods, that benchmark is what LCME will evaluate against.

What Your DCI Response Needs to Address

- In narrative (a), simply describe how students log encounters and skills — no need to discuss centralized vs. departmental structure.
- Review your internal policy on alternate methods: confirm your school has a defined benchmark (whether 20% or another threshold) and that narrative (e) references that school-set benchmark explicitly.

8.7 Comparability of Education/Assessment

MAJOR CHANGE

What Changed

This is the most structurally changed element in Standard 8. Two significant changes: the supporting data table has been removed, and the narrative questions have been reorganized from 3 questions into 5, with different content.

1. Table 8.7-1 ("Actions to Support Comparability") has been eliminated

The previous DCI required a table listing each multi-site course or clerkship and documenting how faculty at distributed sites are informed, how communication occurs, how site leadership receives student performance data, and what actions were taken when disparities were found. This table no longer exists. The information it contained is now distributed across the five narrative questions.

2. Narrative questions restructured: 3 questions → 5 questions

Q	2026–27 DCI Asked...	2027–28 DCI Asks...
a.	Summarize data and information sources used to determine comparability across sites; note centralized vs. director-level determination	How the school communicates learning objectives, assessment methods, and required clinical experiences to faculty and residents at each site (for multi-site disciplines)
b.	Describe individuals/groups responsible for reviewing and acting on comparability data	Summarize data and information sources used to determine comparability across sites (moved from old "a")
c.	Provide an example of an action taken in response to comparability problems	How clerkship/site directors receive site-specific information about student performance and perceptions of quality
d.	— <i>(did not exist)</i>	Describe individuals/groups responsible for reviewing and acting on comparability data (moved from old "b")
e.	— <i>(did not exist)</i>	Provide an example of an action taken in response to comparability problems — now specifically notes "students' perceptions of clerkship quality" alongside completion of experiences and performance/grades

3. Element definition: "medical school" → "medical education program"

Consistent with the DCI-wide institutional language update.

What Your DCI Response Needs to Address

- Do not complete Table 8.7-1 — it no longer exists. The information from it is now addressed through the five narrative questions.
- New narrative (a): Describe specifically how your school communicates learning objectives, assessment methods, and required experiences/skills to faculty and residents at each distributed site.
- New narrative (c): Describe how site directors and clerkship directors receive site-specific student performance data and student perception data.
- Narrative (e): Your comparability action example must now explicitly reference student perception data (course/clerkship quality surveys) — not just completion rates or grades.
- Narratives (b) and (d) retain the substance of the old (a) and (b) — update your response structure accordingly.
- If your school has only one site for a given discipline, note this in narrative (a) and (c) to clarify the element is not applicable for those courses/clerkships.

8.8 Monitoring Student Time

MINOR CHANGE

What Changed

Three targeted updates: two tables renamed for consistency, the element definition updated to clarify shared governance, and narrative question (g) reworded to ask for both the mechanism and frequency of monitoring.

→ *Element definition: "medical school faculty committee" → "faculty committee"; "program's administration" → "education program's administration"*

Continues the DCI-wide language shift; removes "medical school" as the institutional subject.

→ *Table 8.8-2a → Table 8.8-2 (pre-clerkship workload)*

Renaming only; no content change. Update your response to use new table number.

→ *Table 8.8-2b → Table 8.8-3 (clerkship workload)*

Renaming only; no content change. Update your response to use new table number.

→ *Narrative (g): "Describe the frequency with which the curriculum committee monitors..." → "Describe how the curriculum committee monitors...and the frequency with which such monitoring occurs"*

The question now requires both the mechanism (how) and the schedule (how often). Previous responses that described only frequency without describing the monitoring process would be insufficient.

What Your DCI Response Needs to Address

- Update table references: 8.8-2a is now 8.8-2; 8.8-2b is now 8.8-3.
- In narrative (g), describe both how the committee monitors compliance with time policies (the mechanism) and how frequently it does so. A response that only states "reviewed twice per year" without describing the review process is now insufficient.

Summary Action Checklist for the Curricular Affairs Team

Use this checklist when preparing your Standard 8 responses for the 2027–28 DCI.

Governance & Committee Structure (8.1)

- ☐ Confirm committee bylaws explicitly grant authority (not just responsibility) over the curriculum.
- ☐ Verify faculty with medical school appointments constitute a majority of voting members overall and at each meeting.
- ☐ Confirm bylaws define a "general faculty" category and assign at least one voting seat to this category.
- ☐ Update narrative (b) membership category descriptions to distinguish "general faculty" from faculty in leadership roles.

Removed Element (8.2)

- ☐ Do not include an 8.2 response — this element no longer exists.
- ☐ Ensure EPO-to-course-objective linkage evidence is captured in 8.3 narrative (d) instead.

Phase Review & Data Specificity (8.3)

- ☐ Add language to narrative (a) describing how each phase review addresses preparation for the next phase.
- ☐ Name specific data instruments and sources in narrative (a) — replace general process descriptions.

Outcome Thresholds & Supporting Docs (8.4)

- ☐ Update any outcome analysis using the new rolling-average threshold language (do not cite old fixed percentages).
- ☐ Remove NBME Step 1 and Step 2 CK graphs from 8.4 supporting docs — they now belong in 7.1 and 7.2.
- ☐ Confirm narrative (a) addresses evaluation of all EPOs.
- ☐ Update Table 8.4-1: rename first column to "Program Outcome Indicator."

Student Feedback & Monitoring (8.5–8.6)

- ☐ 8.5 narrative (c): describe specific changes made to courses/clerkships — not just student communication.
- ☐ 8.5 narrative (d): present ISA data as a structured summary with percentages and trends.
- ☐ 8.6 narrative (a): simplify — describe the logging process without discussing centralized vs. departmental structure.
- ☐ 8.6 narrative (e): reference your school-set benchmark for alternate methods (not necessarily the 20% example).

Comparability Across Sites (8.7)

- ☐ Do not complete Table 8.7-1 — it has been removed.
- ☐ New narrative (a): describe how learning objectives, assessment methods, and required experiences are communicated to site faculty and residents.
- ☐ New narrative (c): describe how site/clerkship directors receive site-specific student performance and satisfaction data.

- ☐ Narrative (e) example must reference student perception data alongside performance and completion data.
- ☐ Re-map your previous 3 responses to the new 5-question structure (old a→new b, old b→new d, old c→new e).

Student Time Monitoring (8.8)

- ☐ Update table references in your response: 8.8-2a → 8.8-2 and 8.8-2b → 8.8-3.
- ☐ Narrative (g): describe both the monitoring mechanism and the frequency — not just the frequency.

LCME Standard 9 — Teaching, Supervision, Assessment, and Student and Patient Safety

What Changed and What It Means for Our DCI Response

Prepared for the Curricular Affairs Team · 2027-28 DCI Cycle

The Big Picture

Standard 9 has no structural changes - the nine elements and overall organization are unchanged. The substantive updates concentrate in three areas: a major overhaul of the evidence sources for assessment data in Element 9.4 (GQ data replaced by ISA and internal surveys), a significant restructuring of formative assessment documentation in Element 9.7 (new pre-clerkship formative feedback table, ISA data now required by campus), and targeted tightening of narrative questions in 9.1, 9.5, and 9.9 that assume more documentation exists and ask for it explicitly.

Dominant themes across Standard 9

Two cross-cutting shifts define this update. First, LCME is moving away from reliance on AAMC Graduation Questionnaire (GQ) national benchmarks as the primary evidence source for clinical skills data - replacing them with your own ISA data and internal surveys. Second, regional campus data is now explicitly required in several ISA tables (9.7, 9.8) - schools with multi-campus programs that were previously reporting aggregate data will need to break out campus-specific results.

Element Overview

#	Element	Level	Key Change
9.1	Preparation of Resident and Non-Faculty Instructors	MINOR	Narrative questions reorganized (a-d instead of a-c); new question (a) added about campus coverage; old questions shift down one.
9.2	Faculty Appointments	MINOR	One word change: physicians who will teach changed to physicians who teach.
9.3	Clinical Supervision of Medical Students	MINOR	New supporting documentation required: medical school policy/guidelines on student supervision in clinical settings.
9.4	Assessment System	MAJOR	Major table overhaul: GQ data tables replaced with ISA and internal survey data; four tables (previously two); narrative (a) expanded.
9.5	Narrative Assessment	MINOR	Narrative (a) reframed; new narrative (c) added on clerkship compliance; supporting doc updated.
9.6	Setting Standards of Achievement	MINOR	Punctuation correction only.
9.7	Formative Assessment and Feedback	MAJOR	New pre-clerkship formative feedback table added; all ISA tables renumbered; campus-level data now required; narrative (a) reframed.

9.8	Fair and Timely Summative Assessment	MINOR	Academic year references updated; campus-level ISA data now required; ISA item wording clarified.
9.9	Student Advancement and Appeal Process	MINOR	Narrative (d) now assumes recusal policy exists (asks about provisions, not whether it exists); supporting docs split into two items.

Element-by-Element Summary of Changes

9.1 Preparation of Resident and Non-Faculty Instructors

MINOR CHANGE

What Changed

The narrative question structure has been reorganized from three questions (a-c) to four (a-d). A new question (a) has been inserted at the top about campus coverage of resident teaching. The previous questions shift down by one position each.

1. New narrative question (a) — campus coverage

Q	New Question (a) — did not exist in 2026-27 DCI
a.	Do all medical students on all campuses complete one or more required clinical experiences or selectives at an inpatient or outpatient site where residents participate in medical student teaching/supervision? If not, describe how these medical students learn about the expectations and requirements of the next phase of their training. What this means: This is a new gatekeeping question. If any campus or program pathway does not include resident-supervised clinical experiences, LCME wants to know explicitly how those students receive equivalent preparation for the next phase.

2. Previous questions (a), (b), (c) shift to (b), (c), (d)

Previous (2026-27)	New (2027-28)
(a) Policy/formal requirement that residents and others participate in orientation or faculty development	(b) Policy/formal guideline - same content, now question (b)
(b) Evidence that all residents receive clerkship learning objectives, encounters, and orientation to their teaching/assessment role	(c) How the school ensures all residents receive those materials - reframed as process description rather than evidence request
(c) Which institutional individual/office is responsible for ensuring resident preparation	(d) Same question - same content, now question (d)

The content of the shifted questions is largely unchanged. Narrative (c) is reframed as a process description rather than an evidence request.

3. Table instruction: minor clarification

Previous (2026-27)	New (2027-28)
Table instruction: describes how departments ensure learning objectives and assessment orientation have been provided	Clarified: now explicitly says provided to these non-faculty instructors - minor but deliberate addition.

What Your DCI Response Needs to Address

- Confirm whether all students on all campuses complete at least one required experience or selective where residents supervise - answer this explicitly in new narrative (a).
- If any campus pathway lacks resident-supervised experiences, describe the alternative preparation mechanism for narrative (a).
- Rename/renumber your previous (a) to (b), previous (b) to (c), previous (c) to (d).
- Narrative (c) is now about process description (how does the school ensure residents receive materials) rather than requesting evidence directly - adjust your response accordingly.

9.2 Faculty Appointments

MINOR CHANGE

What Changed

One word change in narrative (b):

Previous (2026-27)	New (2027-28)
<i>Narrative (b): physicians who will teach and assess medical students</i>	Narrative (b): physicians who teach and assess medical students (will removed)

Minor change - will removed from the description of physicians. No substantive change to what is being monitored.

What Your DCI Response Needs to Address

- No substantive change required - update the phrasing in your response to say physicians who teach rather than physicians who will teach.

9.3 Clinical Supervision of Medical Students

MINOR CHANGE

What Changed

One new supporting documentation requirement added. No changes to the element definition or narrative questions.

New supporting documentation

New supporting documentation requirement: submit a copy of the medical school policy or guidelines related to medical student supervision in clinical settings.

What Your DCI Response Needs to Address

- Prepare and submit a copy of your institutional policy or guidelines on medical student supervision in clinical settings as supporting documentation.
- If your supervision policy exists only within broader GME or clerkship documents, extract or reference the relevant section.

9.4 Assessment System

MAJOR CHANGE

What Changed

This is the most changed element in Standard 9. The entire supporting data section has been rebuilt around internal data sources rather than AAMC GQ benchmarks. The number of tables increases from two to four, and the content of each changes substantially.

⚠ GQ data is out — internal data is in

All Graduation Questionnaire (GQ) national benchmark data has been removed from Element 9.4. You no longer need to pull or report GQ percentages for clinical skills observation or residency readiness. Instead, LCME now wants your own ISA data, internal clerkship evaluation data, and an internal final-year student survey.

Table-by-table changes:

Table	Previous (2026-27)	New (2027-28)
1	Table 9.4-1 Observation of Clinical Skills Asked for GQ data on percentage of students observed performing history and physical exam in each clerkship. Three academic years, school vs. national %.	Table 9.4-1 Clinical Skills Preparation for Participation in Patient Care Now asks for the types of assessment used for each pre-clerkship clinical skill (history-taking, physical examination, differential diagnosis). No GQ data.
2	Table 9.4-2 Clinical Skills Asked for GQ data on % agreeing they acquired clinical skills required to begin residency. Three academic years, school vs. national %.	Table 9.4-2 I receive adequate clinical skills instruction in the pre-clerkship phase Now asks for ISA data by curriculum year (N/A, disagree, agree) on pre-clerkship clinical skills instruction.
3	(No table)	Table 9.4-3 Observation of Clinical Skills NEW: Internal clerkship evaluation data on % who agree they were observed performing history-taking and physical exam in each required clerkship.
4	(No table)	Table 9.4-4 Clinical Skills Preparation for Residency NEW: Internal final-year student survey data on % who agree/disagree they acquired the clinical skills required to begin residency.

Narrative question updates:

Previous (2026-27)	New (2027-28)
Narrative (a) previously ended at: Describe the methods of assessment and the locations in the pre-clerkship phase of the curriculum where assessment occurs.	Narrative (a) now ends at: Describe the methods of assessment and the locations in the pre-clerkship phase where formative and summative clinical skills assessment occurs. (Formative and summative now explicitly required.)
Narrative (b): comma not present before and the locations	Narrative (b): minor punctuation fix - comma added before and the locations in the clerkship/clinical phase where the assessment occurs.

▶ What Your DCI Response Needs to Address

- Remove GQ data from both tables - it is no longer requested anywhere in Element 9.4.
- Table 9.4-1: map your pre-clerkship clinical skills (history-taking, physical examination, differential diagnosis) to their types of assessment.

- Table 9.4-2: pull ISA data by curriculum year for the item I receive adequate clinical skills instruction in the pre-clerkship phase.
- Table 9.4-3: pull internal clerkship evaluation data on observation rates for history-taking and physical exam across required clerkships.
- Table 9.4-4: prepare or adapt your final-year internal survey to include the item I have acquired the clinical skills required to begin a residency program; report disagree and agree counts and percentages.
- Narrative (a): explicitly address both formative and summative assessment of clinical skills in the pre-clerkship phase.
- Confirm your ISA and final-year internal survey instruments include all four required items; update them if needed.

9.5 Narrative Assessment

MINOR CHANGE

What Changed

Three targeted changes tighten the evidentiary expectations for narrative assessment documentation.

Previous (2026-27)	New (2027-28)
<i>Narrative (a): Summarize the policy/guidelines that describe the circumstances in which narrative descriptions of performance will be provided... Note if these have been centrally developed and approved. How does the school ensure they are provided?</i>	Narrative (a): How does the medical school ensure that they have defined the circumstances in which narrative descriptions of performance will be provided? How does the school ensure they are provided? (Policy summarization removed; focus shifts to whether and how circumstances are defined and enforced.)
<i>No narrative (c)</i>	Narrative (c) ADDED: Note if narrative descriptions of performance have been provided in each required clerkship.
<i>Supporting doc: Sample requirements (i.e., a copy of medical school policy or guideline) related to providing narrative descriptions</i>	Supporting doc: Requirements (i.e., a copy of medical school policy or guideline) - Sample removed. A full policy document is now expected, not an example.

What Your DCI Response Needs to Address

- Narrative (a): frame your response around how the school ensures the circumstances are defined and enforced, not just what the policy says.
- Narrative (c) - NEW: confirm that narrative descriptions are being provided in each required clerkship. If there are gaps, note them.
- Supporting documentation: submit the full policy/guideline, not a sample or excerpt.

9.6 Setting Standards of Achievement

MINOR CHANGE

What Changed

Punctuation correction only: a period was added at the end of item 2 in the element definition. No substantive change.

What Your DCI Response Needs to Address

- No changes required to your response for this element.

9.7 Formative Assessment and Feedback

MAJOR CHANGE

What Changed

Element 9.7 has the most structural changes in Standard 9. A new table for pre-clerkship formative feedback has been inserted at the front, all subsequent tables have been renumbered, ISA item wording has been tightened, and regional campus data is now explicitly required in four ISA tables.

1. Table renumbering and new pre-clerkship formative feedback table

Previous Table	New Table
Table 9.7-1 Mid-clerkship Feedback (listed mechanisms for mid-clerkship feedback)	Now Table 9.7-2 Mid-clerkship Feedback (same content, renumbered)
Table 9.7-2 Pre-clerkship Formative Feedback (mechanisms for formative feedback in pre-clerkship courses)	Now Table 9.7-1 Pre-clerkship Formative Feedback - MOVED TO FRONT and content unchanged, but new columns added if applicable
Table 9.7-3a (ISA: amount of pre-clerkship formative feedback)	Now Table 9.7-3 same ISA item, wording trimmed; campus-level data now required for multi-campus schools
Table 9.7-3b (ISA: quality of pre-clerkship formative feedback)	Now Table 9.7-4 same ISA item, wording trimmed; campus-level data now required
Table 9.7-3c (ISA: amount of clerkship formative feedback)	Now Table 9.7-5 same ISA item; campus-level data now required
Table 9.7-3d (ISA: quality of clerkship formative feedback)	Now Table 9.7-6 same ISA item; campus-level data now required

2. ISA item wording changes

Previous (2026-27)	New (2027-28)
Table 9.7-3a item: The amount of formative feedback in the pre-clerkship phase is sufficient to allow me to assess how I am meeting the expectations in the courses of this phase of the medical education program.	Table 9.7-3 item: The amount of formative feedback in the pre-clerkship phase is sufficient to allow me to assess how I am meeting the expectations in the courses of this phase. (medical education program removed from end)
Table 9.7-3b item: The quality of formative feedback in the pre-clerkship phase allows me to identify areas in which I need to improve as I progress through this phase of the curriculum.	Table 9.7-4 item: The quality of formative feedback in the pre-clerkship phase allows me to identify areas in which I need to improve. (as I progress through this phase of the curriculum removed)

These wording changes are minor but could affect whether your ISA instrument items match the DCI. Verify your survey instrument uses the new wording.

3. Narrative and supporting documentation changes

Previous (2026-27)	New (2027-28)
Narrative (a): Describe how the curriculum governance process and associated formal policy/guideline ensure that students will receive mid-course and mid-clerkship feedback.	Narrative (a): Describe how the school ensures that students receive mid-course and mid-clerkship feedback. Note if there is a formal policy/guideline specifying this requirement. (Removes the requirement that governance process be the mechanism; allows other methods; makes policy optional.)
Supporting doc: Any curriculum committee-approved policy or guideline that medical students receive formative feedback by at least	Supporting doc: The formally approved policy or guideline...if available. (Any removed; if available added - acknowledges schools may use other mechanisms rather than formal policy.)

the mid-point of courses and clerkships of four weeks or longer.

What Your DCI Response Needs to Address

- Update your table numbering throughout: old 9.7-1 is now 9.7-2; old 9.7-2 is now 9.7-1 (moved to front); old 9.7-3a through 3d are now 9.7-3 through 9.7-6.
- If your school has regional campuses, break out ISA data by campus in Tables 9.7-3, 9.7-4, 9.7-5, and 9.7-6.
- Verify your ISA instrument wording matches the updated table items (the phrase medical education program was removed from one item; the phrase as I progress through this phase of the curriculum was removed from another).
- Narrative (a): you no longer need to frame mid-course/mid-clerkship feedback as a governance process requirement. Describe how the school ensures it happens - whether through policy, process, or monitoring.
- Supporting doc: submit your policy or guideline if one exists; if feedback is ensured through process rather than formal policy, note this in narrative (a) instead.

9.8 Fair and Timely Summative Assessment

MINOR CHANGE

What Changed

Mostly administrative updates. The most significant practical change is the addition of campus-level ISA data requirements for multi-campus schools, consistent with the same update in 9.7.

Previous (2026-27)	New (2027-28)
Academic year data columns: AY 2023-24, 2024-25, 2025-26	Updated to: AY 2024-25, 2025-26, 2026-27
Table 9.8-2 ISA item: Pre-clerkship summative assessments are pertinent to the course objectives and content taught in the courses of this phase of the medical education program.	Table 9.8-2 ISA item: Pre-clerkship course summative assessments are pertinent to the course objectives and content taught in that course. (Tightened to course-level, removed phase-level framing.)
ISA tables: aggregate data only	ISA tables: If the medical school has one or more regional campuses, provide the data by campus. (Four ISA tables now require campus-level breakdown.)
Narrative (b): grades within six weeks during the 2025-26 academic year	Narrative (b): grades within six weeks during the 2026-27 academic year
Narrative (d): assessments matched/did not match the pre-clerkship course learning objectives	Narrative (d): assessments matched/did not match the learning objectives (pre-clerkship course removed - now applies to all phases)

What Your DCI Response Needs to Address

- Update academic year references throughout to 2024-25 through 2026-27.
- If your school has regional campuses, provide campus-level ISA data in all applicable 9.8 tables.
- Confirm your ISA instrument wording matches the updated Table 9.8-2 item.
- Update narrative (b) academic year reference to 2026-27.

9.9 Student Advancement and Appeal Process

MINOR CHANGE

What Changed

Two targeted changes - one in the narrative questions and one in supporting documentation - that signal a higher expectation for recusal policy formalization.

⚠ Recusal policy is now assumed to exist

The previous DCI asked whether a recusal policy is in place. The new DCI asks you to describe the provisions of the recusal policy that is in place - treating its existence as a given. If your school does not currently have a formal recusal policy for the promotions committee, establishing one should be a priority before your survey visit.

Previous (2026-27)	New (2027-28)
<i>Narrative (d): If the promotions committee includes course/clerkship directors or clinical faculty, describe whether a recusal policy is in place for committee members who may have a conflict of interest.</i>	Narrative (d): describe the provisions of the recusal policy that is in place. (Whether changed to the provisions of - LCME now assumes a recusal policy exists and asks about its content, not its existence.)
<i>Supporting doc item 2: Policies and procedures for disciplinary action and due process AND the charge to or the terms of reference for the medical student promotions committee(s). (Both in one item.)</i>	Supporting doc item 2: Policies and procedures for medical student disciplinary action and due process. (One item only.) Supporting doc item 3 (NEW): Policies and procedures that state the charge to and membership composition of the medical student promotion committee(s) and its operating processes, including management of conflicts of interest for committee members.

▶ What Your DCI Response Needs to Address

- Narrative (d): do not frame your response around whether a recusal policy exists - assume it does and describe its specific provisions. If your school does not yet have a formal recusal policy, that is a gap to address before the survey visit.
- Supporting documentation: item 2 is now solely about disciplinary action and due process.
- Supporting documentation item 3 (NEW): prepare a separate document (or clearly delineated section) with the charge, membership composition, and operating processes of the promotions committee, explicitly addressing how conflicts of interest are managed.
- If your promotions committee policies and procedures were previously combined with disciplinary action policies in one document, consider whether they need to be separated or cross-referenced to address both new items clearly.

Summary Action Checklist for the Curricular Affairs Team

Use this checklist when preparing your Standard 9 responses for the 2027-28 DCI.

Resident and Non-Faculty Instructor Preparation (9.1)

- ☐ Add new narrative (a): confirm whether all students on all campuses complete required experiences/selectives where residents supervise; if not, describe the alternative.
- ☐ Renumber your previous (a) to (b), previous (b) to (c), previous (c) to (d).
- ☐ Update narrative (c) framing: describe the process for ensuring residents receive materials, rather than listing evidence.

Clinical Supervision Supporting Documentation (9.3)

- ☐ Prepare and submit a copy of your medical school policy or guidelines related to medical student supervision in clinical settings.

Assessment System Data Sources (9.4)

- ☐ Remove all GQ data from Element 9.4 - no longer requested.
- ☐ Table 9.4-1 (new): map pre-clerkship clinical skills (history-taking, physical exam, differential diagnosis) to types of assessment.
- ☐ Table 9.4-2 (new): pull ISA data by curriculum year for the pre-clerkship clinical skills instruction item.
- ☐ Table 9.4-3 (new): pull internal clerkship evaluation data on observation rates for history-taking and physical exam.
- ☐ Table 9.4-4 (new): ensure your final-year internal survey includes the residency-readiness clinical skills item; report disagree/agree data.
- ☐ Narrative (a): address both formative and summative assessment of clinical skills in the pre-clerkship phase.
- ☐ Confirm/update your ISA and final-year survey instruments to include all four required items.

Narrative Assessment (9.5)

- ☐ Narrative (a): reframe around how circumstances are defined and enforced, not a policy summary.
- ☐ Narrative (c) - NEW: confirm and document that narrative descriptions are being provided in each required clerkship.
- ☐ Supporting documentation: submit the full policy, not a sample.

Formative Assessment Table Updates (9.7)

- ☐ Renumber tables: old 9.7-1 becomes 9.7-2; old 9.7-2 becomes 9.7-1 (now first); old 9.7-3a through 3d become 9.7-3 through 9.7-6.
- ☐ If your school has regional campuses, add campus-level ISA data to Tables 9.7-3, 9.7-4, 9.7-5, and 9.7-6.
- ☐ Verify ISA instrument wording matches updated table items (two items were shortened).
- ☐ Narrative (a): describe how the school ensures feedback happens - policy is no longer required as the mechanism.
- ☐ Submit formal policy if it exists; if not, explain the process used instead.

Summative Assessment and Academic Year Updates (9.8)

- ☐ Update all academic year references to AY 2024-25, 2025-26, 2026-27.
- ☐ If your school has regional campuses, add campus-level ISA data to applicable 9.8 tables.
- ☐ Update narrative (b) academic year reference to 2026-27.
- ☐ Confirm your ISA instrument matches the updated Table 9.8-2 item wording.

Student Advancement and Recusal Policy (9.9)

- ☐ Narrative (d): describe the provisions of your recusal policy - assume it exists and detail how it works.
- ☐ If a formal recusal policy does not exist, establish one before the survey visit.
- ☐ Supporting doc item 2: retain the disciplinary action and due process document.
- ☐ Supporting doc item 3 - NEW: prepare a document (or section) with the promotions committee charge, membership composition, operating processes, and conflict-of-interest management.